

6
Headache days

6
Headache days

7
Avg worst /10

7
Avg worst /10

7
Peak /10

7
Peak /10

0
Med doses

0
Med doses

Country	Very worst /10
Spain	10
Italy	10
France	10
Germany	10
UK	10
China	10



Each bar = one logged day, colored by worst pain (teal → red).

[illegible]

2025-06-08	Mon	7	Temple (7)	12h	nausea (moderate), photophobia, phonophobia, vomiting, aura, aggravated by activity	Visual aura, vomiting, right throbbing pain.
2025-06-09	Tue	0				No headache logged.
2025-06-10	Wed	0				No headache logged.
2025-06-11	Thu	0				No headache logged.
2025-06-12	Fri	0				No headache logged.
2025-06-13	Sat	0				No headache logged.
2025-06-14	Sun	7	Temple (7)	12h	nausea (moderate), photophobia, phonophobia, vomiting, aura, aggravated by activity	Visual aura, vomiting, right throbbing pain.

ICHD-3 Classification Analysis

Deterministic aggregation of the logged pattern against published ICHD-3 criteria, for clinician review. Diagnoses requiring a clinical test (e.g. indomethacin trial, imaging, nerve block) are capped at “needs clinical test” and are never auto-concluded from the diary. Only diagnoses with at least a partial signal are shown.

Migraine without aura ICHD-3 1.1

Criteria met

✓

A. ≥5 attacks (got 6)

✓

B. Attacks last 4-72h (untreated)

✓

C. ≥2 of unilateral, pulsating, moderate/severe, aggravated by activity (got 4)

✓

D. ≥1 of: nausea/vomiting, OR photophobia+phonophobia

✓

E. Not better accounted for by another ICHD-3 dx

Migraine with aura ICHD-3 1.2

Criteria met

✓

A. ≥2 attacks (got 2)

✓

B. Aura: fully reversible, with ≥2 of: visual/sensory/speech/motor/brainstem/retinal

✓

C. Aura precedes headache by ≥5 min (got 2)

✓

D. Not better accounted for by another ICHD-3 dx

Decision support, not a diagnosis. This report is a deterministic aggregation of published ICHD-3 criteria from self-logged diary data; it reads only structured fields, never the free-text notes, so multilingual notes can never change a verdict. Every primary-headache criterion ends with “not better accounted for by another ICHD-3 diagnosis,” which the engine assumes unless a clinician sets otherwise. Confirm with a clinician.

Cause Finder — beyond the primary differential

Possible secondary / fixable causes and cranial neuralgias the logged pattern fits, with the test that confirms or excludes each. Decision support for clinician review — not a diagnosis.

Sleep-related (incl. sleep apnea)

[Discuss](#)

✓ headaches present on waking are often sleep-related

Test: A sleep study (especially if you snore, wake unrefreshed, or have morning headaches). · See: [Sleep clinic](#) / [GP referral](#)

Worth considering only if mornings are your worst time / you have sleep symptoms.

Tests worth considering

► Sleep study. Morning/sleep-linked headaches can be sleep-apnea related.